The Constraint

Why the current model can't reach the board's target

MedFlex today vs. what the board requires

- Current: **$14M revenue**, **8 coordinators**, 4.2h average fill time
- Board target: **$200M in 24 months** — roughly 14× growth
- At current productivity: **112 coordinators needed** to match that volume manually

The bottleneck isn't effort — it's architecture.

Reaching the board target requires the coordinator role to shift from doing every match to supervising agents that handle standard cases autonomously. That is an infrastructure problem, not a hiring problem.

What's Breaking Today

Three operational failures that compound as volume grows

Process design gaps — not coordinator mistakes

- **Speed loss** — Hospitals submit to multiple agencies simultaneously. At 4.2h fill time, MedFlex loses contracts before a proposal is even sent.
- **Throughput ceiling** — 8 coordinators × 120 decisions/day is a hard cap. Demand spikes create queues; queues create more delay; delay compounds competitive loss.
- **Tribal knowledge lock** — Senior coordinators carry undocumented matching logic. New hires take longer and produce inconsistent results. When people leave, the knowledge leaves.

Hidden problems surfaced in discovery

- **Free-text intake** — Every hospital email is manually parsed before matching begins. Hidden latency step.
- **12% no-show rate** — Nurses are notified by SMS; silence is treated as acceptance. MedFlex finds out at shift time.
- **Double-booking risk** — Same nurse submitted to multiple hospitals simultaneously. No automated conflict resolution at scale.

The Solution

Three agents, existing systems unchanged

Agent relay: intake → matching → confirmation

- **Agent 1 — Reader**

  Converts unstructured hospital emails into structured shift records. 5-minute coordinator preview before proceeding — catches silent misreads before they cascade.

- **Agent 2 — Matchmaker**

  Scores each candidate against credentials, availability, proximity, and hospital history. High-confidence (>85%) proposals go automatically. Mid-confidence proposals go with a 30-minute coordinator recall window. Low-confidence cases are escalated to a human with ranked options.

- **Agent 3 — Follow-up**

  Requests explicit nurse confirmation after hospital accepts. Escalates silence before shift day. Triggers emergency re-match if no-show detected.

Coordinator role: exception management, not routine matching.

- **What stays the same:** ServiceNow queue, hospital email/portal/phone channels, nurse SMS/email comms

How Risk Is Managed

Incremental trust-building, not a big-bang cutover

Four safeguards built into the design

- **Phased rollout** — Phase 1 handles only the highest-confidence 50% of daily volume. Coordinators review everything else. Full autonomy is earned, not assumed.
- **Coordinator recall window** — Every mid-confidence automated proposal can be pulled back within 30 minutes. Oversight without blocking speed.
- **Nurse reservation protocol** — When a nurse is proposed to a hospital, a 15-minute soft lock prevents the same nurse being offered elsewhere simultaneously. Eliminates double-booking at scale.
- **30-day calibration gate** — Full automation of high-confidence cases is not activated until 30 days of shadow review confirms the mismatch rate stays below 3%.

Why this is different from the two prior AI projects

- Previous chatbot: tried to change how hospitals submit requests. This engagement keeps hospital channels unchanged.
- Previous recommendation engine: coordinators didn't trust it. This engagement starts with high-confidence automation only, keeping coordinators visible and in control for all borderline cases.

The Ask

Phase 1 approval to establish the ROI signal

Phase 1 — 8 weeks, measurable outcome

- **Scope:** Intake parsing agent + high-confidence matching agent + coordinator dashboard
- **Target:** 50% of daily proposals handled autonomously; fill time under 1 hour
- **Success signal:** Coordinator time freed × cost savings; mismatch rate tracked separately from hospital preference rejection

What's needed from IT

- ServiceNow API access (read queue, write structured ShiftRequest back)
- Read access to nurse database (credentials, availability, history)
- Confirmation on availability data freshness — current assumption: <24h lag
- SMS/email gateway configuration for nurse confirmation notifications

Decision needed: approve Phase 1 build to prove ROI before Phase 2 investment.

Phase 2 (weeks 9-16) and Phase 3 (weeks 17-24) are scoped and ready — but contingent on Phase 1 results.